

Management

Information, Explanation and Advice for the Patient

Patients who are otherwise healthy should be informed that molluscum contagiosum is a benign viral skin infection that typically resolves spontaneously within 6–12 months. Although harmless, it is contagious and can spread locally or to others.

To minimize local spread, patients should avoid shaving, waxing, or scratching the affected areas. Transmission occurs through direct skin-to-skin contact and fomite exposure (e.g., towels, bedding), so individuals should cover lesions and refrain from sharing personal items. While swimming, lesions should be covered to reduce transmission risk.

The protective benefit of condoms is limited, particularly when lesions are present on uncovered areas such as the thighs or pubic region.

Therapy

In immunocompetent individuals, watchful waiting without active treatment is a reasonable approach (GRADE 2, A), as the condition often resolves on its own. However, many patients—especially those with sexually acquired lesions—opt for treatment due to cosmetic concerns, discomfort, or fear of transmission.

Indications for active therapy include: - Patient preference

- Extensive or persistent lesions
- Cosmetic concerns
- Risk of autoinoculation or transmission
- Symptoms such as pruritus, inflammation, or secondary infection
- Risk of scarring

While treatment may accelerate clearance, it must be weighed against potential discomfort and adverse effects, particularly in sensitive genital areas. Patients

should be advised that new lesions may appear even after visible ones are cleared, possibly requiring repeated interventions.

Associated eczema should be managed with emollients and, if needed, a topical corticosteroid—especially when pruritic—to reduce scratching and subsequent autoinoculation (GRADE 1, C).

Treatment Selection

There is limited evidence comparing the efficacy of various treatments. The choice of therapy depends on several factors:

- Number and location of lesions
- Availability, cost, and ease of treatment
- Efficacy and side-effect profile
- Patient and clinician preference and experience
- Immune status of the patient

Proposed mechanisms of action include:

- Destruction of infected epidermal cells

- Stimulation of local immune response
- Direct antiviral effects

Physical Treatments

Cautery

Cautery is simple, rapid, and provides immediate lesion removal (GRADE 1, D). When applied lightly to the raised portion of the lesion, it causes minimal damage, with healing typically occurring within days and little to no scarring. Pain can be reduced with prior application of topical anesthetic (e.g., 5% lidocaine-prilocaine cream).

Curettage

Curettage is an effective alternative (GRADE 1, C) and may offer more definitive removal than cautery. However, it is more traumatic and painful, making it less suitable for genital skin (GRADE 1, C).

Manual Expression

Squeezing lesions with forceps or puncturing them with a sterile instrument

(e.g., clean needle or cocktail stick) may be effective, though no formal studies support this approach.

Cryotherapy

Cryotherapy with liquid nitrogen is commonly used in sexual health and dermatovenereology clinics (GRADE 1, C), despite a lack of published studies specifically on genital molluscum. Two studies in non-genital molluscum showed that weekly cryotherapy achieved over 80% clearance after 4–6 weeks—comparable to topical potassium hydroxide.